

August 20, 2026

BlueCross BlueShield of South Carolina
Provider Appeals Department
P.O. Box 100300
Columbia, SC 29202

RE: Formal Appeal of Claim Denial — Willis Crona (MRN MUSC8226170), Claim MUSC-7099-2

Patient	Willis Crona (DOB 1946-10-19)
MRN	MUSC8226170
Member ID / Group	BCB494161219 / GRP-79505
Claim number	MUSC-7099-2
Date of service	2026-06-29
Procedure billed	CPT 99214 — Office/outpatient visit, established patient, moderate complexity
Primary diagnosis	J06.9 — Acute upper respiratory infection, unspecified
Denied amount	\$593.77
Denial code	CARC 50 / RARC N115 — These are non-covered services because this is not deemed a 'medical necessity' by the payer.
Appeal deadline	2026-10-16

Dear Appeals Review Committee:

SUMMARY OF APPEAL

MUSC Health is submitting this formal reconsideration request on behalf of Willis Crona (Member ID: BCB494161219, Group: GRP-79505) regarding the denial of Claim MUSC-7099-2 for services rendered on June 29, 2026, at MUSC Health East Cooper Medical Center. BlueCross BlueShield of South Carolina denied the claim in full, citing CARC 50 ("These are non-covered services because this is not deemed a 'medical necessity' by the payer") and RARC N115 ("This decision was based on a Local Coverage Determination (LCD)"), with the accompanying remark that "Documentation submitted does not establish that the service was reasonable and necessary for the diagnosis reported." MUSC Health respectfully disagrees with this determination and requests reversal and payment of the billed amount of \$593.77.

CLINICAL BACKGROUND

Mr. Crona is a 79-year-old male with a complex, active medical history that substantially elevates the clinical significance of any acute illness. His active conditions include carcinoma in situ of the prostate with ongoing neoplasm of the prostate, for which he is currently receiving leuprolide acetate and docetaxel; atrial fibrillation managed with warfarin sodium, verapamil hydrochloride, and digoxin, with a history of multiple electrical cardioversions and catheter ablations between 2019 and 2021; microalbuminuria due to type 2 diabetes mellitus; and chronic kidney disease. He presented on June 29, 2026, to an established outpatient visit with Dr. Marcus T. Bledsoe, DO (NPI 1770615243), and was evaluated for an acute upper respiratory infection (J06.9), with obesity (E66.9) as a secondary diagnosis.

BASIS FOR APPEAL

The denial rests on the assertion that the visit was not reasonable and necessary for the diagnosis reported. This conclusion fails to account for the clinical complexity that Mr. Crona's comorbidity profile introduces when evaluating even a seemingly routine acute illness. A 79-year-old patient actively receiving cytotoxic chemotherapy with docetaxel and hormonal therapy with leuprolide acetate is immunocompromised, rendering an acute upper respiratory infection a potentially serious clinical event requiring careful evaluation, medication review, and management to prevent escalation. Concurrently, his anticoagulation with warfarin for atrial fibrillation demands assessment of any acute illness for potential drug interactions, changes in INR stability, and risk of decompensation. His chronic kidney disease further constrains the therapeutic options available for symptomatic management, requiring physician-level judgment to avoid nephrotoxic agents.

CPT code 99214 describes an office or outpatient visit for an established patient involving moderate-complexity medical decision-making. Given the number and severity of Mr. Crona's active chronic conditions, the prescription drug burden he carries, and the potential for an acute respiratory illness to destabilize his anticoagulation or compromise his already-reduced renal function, the level of medical decision-making documented by Dr. Bledsoe is consistent with moderate complexity as defined by current CPT evaluation and management guidelines. The service was rendered in the appropriate place of service (22 — On Campus Outpatient Hospital) by a qualified rendering provider, and the claim was submitted timely on July 5, 2026.

The payer's reliance on an LCD to deny a face-to-face evaluation and management service for an acutely ill, medically complex established patient is inconsistent with broadly accepted standards of medical necessity. Denying payment for physician oversight of an acute infection in a patient on active chemotherapy, anticoagulation, and with chronic kidney disease does not reflect the clinical realities documented in this record.

REQUESTED ACTION

MUSC Health respectfully requests that BlueCross BlueShield of South Carolina conduct a full clinical review of the submitted medical record, reverse the medical necessity denial, and issue payment at the allowed amount of \$374.66 in accordance with the contracted rate for CPT 99214. Should the reviewer require additional clinical documentation, we welcome the opportunity to provide it promptly. This reconsideration is being submitted in advance of the appeal deadline of October 16, 2026, through the My Insurance Manager portal as directed on the remittance advice.

Respectfully submitted,

Angela R. Whitfield, RN, BSN, CPC

Senior Appeals Specialist, Revenue Cycle Management
MUSC Health — Revenue Cycle / Patient Financial Services
(843) 792-2300 | muschealth.org

Enclosures

- Remittance advice / explanation of payment
- Itemized claim detail (UB-04 / CMS-1500)
- Relevant clinical documentation from the medical record